

1. Administrative & Study Identification

Full Study Title: KonTRASt-03: A Phase Ib/II, Multicenter, Open-label Platform Study of JDQ443 With Select Combinations in Patients With Advanced Solid Tumors Harboring the KRAS G12C Mutation **Short Title/Acronym:** KonTRASt-03 **Protocol Number:** CJDQ443E12101 **NCT Number:** NCT05358249 **Sponsor Name:** Novartis **Investigational Product:** JDQ443 (Opnurasib) in combination with partner therapies (e.g., cetuximab) **Phase of Development:** Phase 1, Phase 2 **Medical Monitor:** Novartis Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the safety and tolerability of JDQ443 in combination with selected partner therapies by assessing the incidence and severity of dose limiting toxicities (DLTs), adverse events (AEs), serious adverse events (SAEs), and the frequency of dose interruptions and reductions during dose escalation. **Secondary Objectives:** To evaluate anti-tumor activity including Objective Response Rate (ORR), Disease Control Rate (DCR), and Duration of Response (DoR) by local review per RECIST 1.1.

2.2 Background & Rationale To address the gap in durable treatment responses for patients with advanced solid tumors harboring the KRAS G12C mutation (such as non-small cell lung cancer [NSCLC] and colorectal cancer [CRC]). JDQ443 will be considered a "backbone" treatment in this trial and combined with selected therapies, or "partner(s)". After dose escalation, treatment arms that reach a maximum tolerated dose / recommended dose and are determined to be safe may, but are not required to, proceed to Phase II to further explore safety, tolerability, and anti-tumor activity.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Adaptive platform with multiple combination arms) **Blinding:** Open-label **Randomization:** N/A **Trial Status:** Active, not recruiting (ACTIVE_NOT_RECRUITING)

3.2 Planned Enrollment & Duration Target \$N\$: 74 patients **Number of Sites:** Multicenter (Global) **Estimated Study Start:** 24-Oct-2022 **Estimated Primary Completion:** 30-Nov-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria Dose Escalation: 1. Patients with advanced (metastatic or unresectable) KRAS G12C mutant solid tumors who have received standard of care therapy or are ineligible to receive such therapy. **Phase II:** 2. Patients with advanced (metastatic or unresectable) KRAS G12C mutant non-small cell lung cancer who have received platinum-based chemotherapy regimen and immune checkpoint inhibitor therapy, unless patient was ineligible to receive such therapy. 3. Patients with advanced (metastatic or unresectable) KRAS G12C mutant colorectal cancer who have received fluoropyrimidine-, oxaliplatin-, and irinotecan-based chemotherapy, unless patient was ineligible to such therapy. **All patients:** 4. ECOG performance status of 0 or 1. 5. Patients must have a site of disease amenable to biopsy and be a candidate for tumor biopsy according to the treating institution's guidelines.

4.2 Exclusion Criteria 1. Tumors harboring driver mutations that have approved targeted therapies, with the exception of KRAS G12C mutations. 2. Prior treatment with a KRAS G12C inhibitor is excluded for patients in a subset of groups in Phase II. 3. Active brain metastases, including symptomatic brain metastases or known leptomeningeal disease. 4. Clinically significant cardiac disease or risk factors at screening. 5. Insufficient bone marrow, hepatic or renal function at screening. Other protocol-defined inclusion/exclusion criteria may apply.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: JDQ443 (opnurasib) serves as the "backbone" treatment, administered in continuous 28-day cycles. It is combined with partner therapies (e.g., cetuximab) based on the assigned treatment arm. Doses are determined during the dose-escalation phase. **Route of Administration:** Oral (JDQ443) and Intravenous (cetuximab). **Duration of Treatment:** Continued until disease progression, unacceptable toxicity, or subject withdrawal.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for advanced cancer and symptom management. **Prohibited:** Concurrent investigational drugs, other anticancer therapies, and medications known to be strong CYP3A inhibitors/inducers that may interfere with the pharmacokinetics of the study drugs.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
1	Day -28 to 0	Informed Consent, Mutational Status Confirmation (KRAS G12C), Baseline Imaging,

Labs, ECG | | **Baseline** | Day 1 | Enrollment, First Dose of JDQ443 +/- Partner Therapy | | **Interim** | Every 3-4 Weeks | Safety Labs, PK/PD Assessment, Tumor Imaging, DLT Assessment in first 28 days | | **End of Study** | At Progression | Final Vital Signs, Exit Interview, IP Accountability, Survival Follow-Up |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoints 1. **Dose escalation:** Incidence and severity of dose limiting toxicities (DLTs) of each combination treatment. A dose-limiting toxicity (DLT) is defined as an adverse event or abnormal laboratory value that is not primarily related to disease, disease progression, intercurrent illness/injury, or concomitant medications that occurs within the first 28 days of study treatment and meets a defined criteria. 2. **Dose escalation:** Incidence and severity of adverse events (AEs) and serious adverse events (SAEs) by treatment. All information obtained on AE will be displayed by treatment group. Summary tables will include only AEs that started/worsened during the cycles of treatment (treatment-emergent AEs). 3. **Dose escalation:** Frequency of dose interruptions and reductions, by treatment. The number of patients with dose adjustments (reductions and interruptions) will be summarized by treatment group.

7.2 Secondary & Exploratory Endpoints 1. **Dose escalation and Phase II:** Objective Response Rate (ORR) by local review per RECIST 1.1. 2. **Dose escalation and Phase II:** Disease Control Rate (DCR) by local review per RECIST 1.1. 3. **Dose escalation and Phase II:** Duration of Response (DoR) by local review per RECIST 1.1.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via regular clinical visits, laboratory monitoring, and ECGs. **Serious Adverse Events (SAEs):** Evaluated and reported promptly to the sponsor and regulatory authorities per local guidelines. **Data Monitoring Committee (DMC):** An internal safety review committee will evaluate DLTs and toxicity data to guide dose-escalation and expansion decisions.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all patients who receive at least one dose of study medication); Full Analysis Set for efficacy endpoints. **Sample Size Justification:** Target enrollment of approximately 74 patients, powered adaptively based on the number of partner arms that reach the recommended Phase II dose. **Handling of Missing Data:** Standard imputation methods for missing or partial dates.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Regular onsite and remote monitoring by the Sponsor to ensure data integrity and protocol adherence. **Audit**

Trail: Secure Electronic Data Capture (EDC) system with full audit trails for eCRF locking and data clarification forms.

11. Study Identifiers & Governance

Primary ID: {{CJDQ443E12101}} **Registry Number:** {{NCT05358249}}

Sponsor/Lead Organization: {{Novartis}} **Study Title:** {{Platform Study of JDQ443 in Combinations in Patients With Advanced Solid Tumors Harboring the KRAS G12C Mutation}} **Official Regulatory Title:** {{KonTRASt-03: A Phase Ib/II, Multicenter, Open-label Platform Study of JDQ443 With Select Combinations in Patients With Advanced Solid Tumors Harboring the KRAS G12C Mutation}}

12. Clinical Framework (Oncology Specific)

Primary Condition: {{KRAS G12C Mutant Solid Tumors, Non-Small Cell Lung Cancer, Colorectal Cancer, Carcinoma}} **Diagnosis**

Threshold: {{Histologically confirmed advanced/metastatic solid tumor with KRAS G12C mutation}} **Medical Methodology:** {{Targeted Combination Therapy}} **Optimization Target:** {{Maximum Tolerated Dose (MTD), Safety, and Objective Response Rate (ORR)}}

13. Study Procedures & Methodology

Management Pathway: {{Dose Escalation followed by Dose Expansion Phase II}} **Education Protocol (HCP):** {{Protocol training, DLT

criteria management, and Investigator Meetings}} **Education**

Protocol (Patient): {{Informed consent process and drug administration diaries}} **Quality Audit Mechanism:** {{Routine Source Data Verification (SDV) and central data monitoring}}

14. Observation & Follow-Up Schedule

Total Duration: {{Until disease progression or unacceptable toxicity}} **Onsite Visit Cadence:** {{Regularly scheduled per treatment cycle (e.g., every 3-4 weeks)}} **Remote Monitoring**

Frequency: {{As needed per institutional guidelines}} **Checklist**

Review Interval: {{Prior to the start of each new treatment cycle}}

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					
Statistician	[Name]	_____	[DD-MMM-YYYY]					